

THE TREATMENT OF MENSTRUAL DISORDERS IN GENERAL PRACTICE

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THE general practitioner can expect about 10 per cent of his patients to complain of menstrual difficulties. Although dysmenorrhea is the most common, excessive bleeding and irregular cycles are also quite frequent. Therapy for these conditions has been unsatisfactory. Every year there appears a large number of papers in which every new drug or hormone has been tried with varying degrees of success. Relief of the pain with sedation, curettement of the endometrium for irregularity, or excirpation of the uterus for bleeding does nothing to correct the abnormal physiology responsible for the patient's complaint.

In recent years attention has been focused on the sex hormones and the other steriods since the biochemists have been able to isolate and synthesize pure potent preparations. They have all been extensively used but the results have been disappointing. Dr. Emil Novak summarized their effectiveness for this Society last year when he pointed out that their greatest indication was in menopause.

In 1939 Foster and Thornion¹ reported 50 cases of menstrual irregularities treated with thyroid therapy. Their results were far better than any published before or since. This present report on 143 cases is presented in confirmation of their work, and outlines an inexpensive plan of management which produces gratifying results.

There is no substitute for a routine history and physical examination including examination of the pelvis. If the hymen is intact, a rectal examination will suffice. A blood count, sedimentation rate and complete urine analysis are the minimum laboratory requirements. Any pathology found should be corrected because the menstrual cycle is often influenced by the general health of the patient. I have abandoned the use of the Basal Metabolism Test except for my colleagues. Too many patients can not relax; and as a result one measures the basal plus an unknown quantity of nervous energy. I have seen the reading drop 40 per cent in 5 minutes just by getting the patient to relax.

For some years I sought a substitute for the basal metabolic rate and finally adopted the use of the Basal Axillary temperature. Over 1000 cases have been published² in which both basal temperature and basal metabolism were determined and it was found that basal temperature gave a better index of the clinical findings in the patient. During the war Dr. Joseph C. Ehrlich of Phoenix and I collected simultaneous oral, axillary and rectal temperatures. Although the results are unpublished as yet, they indicate that if the thermometers are left in place 10 minutes, the oral and axillary temperatures are about the same, while the rectal temperature is a little higher. Since a mild upper respiratory cold will elevate the oral temperature, the axillary temperature has been adopted as an index of thyroid function. The temperature of the female varies with the menstrual cycle, reaching a peak a few days before the onset of the menstruation and a low point at the time of ovulation. For that reason the temperature is taken on the second and third day of the menstrual period of each month. The normal basal temperature at that time is thought to range from approximately 97.8 to 98.2 F. An overdosage of thyroid will cause an elevation above this level and symptoms of hyperthyroidism will appear.

In the majority of patients with menstrual irregularities no evidence of endometriosis, ovarian cysts, fibroids, cervical polyps or other pathology can be found. It is this functional group that the general practitioner can manage quite successfully. Practically all of these patients will run low basal metabolic rates, if they are relaxed, and low basal temperatures. The results of thyroid therapy can best be illustrated by the accompanying table.

It can be seen that only 5 cases out of 48 with dysmenorrhea failed to get some relief while 35 were completely cured of their pain. Many cases of cramps are due to the passage of clots, and thyroid therapy often corrects this condition. Three of the five failures had an additional diagnosis at a later date, which were confirmed by

TABLE I
Showing the Effect of Thyroid Therapy
on Menstrual Disorders

	Dysmenorrhea	Excessive Bleeding	Irregular Cycle
Total Cases	48	50	45
Cured	35	46	41
Improved	8	2	2
No Change	5	2	2

surgery. Two patients had ovarian cysts and one had endometriosis. It is not surprising they failed to respond to medical management.

In the 50 cases of excessive bleeding are included those who flowed too many days, and those who flooded at the time of menstruation. All age groups are represented including the menopause. It can be seen that there were only two failures and that two were improved, while 46 resumed periods with a normal flow. In the last two years I have curetted only one case for functional uterine bleeding and she was completely bled out upon entry to the hospital at the first visit. Therapy on functional bleeding can be illustrated by the case of a 39-year-old woman whose periods had been regular until the past 6 months, during which time she was flowing too many days and too heavily. Pathology could not be found. Her basal metabolic rate was low and her temperatures were low. She was given thyroid therapy and for three months had regular periods of five days each. She was not seen again, but five months later consulted another doctor for the same symptoms. A curettage failed to give relief and normal endometrium was reported. She was then subjected to a hysterectomy and the pathologist reported a normal uterus. This case was clearly one of functional bleeding which responded to medical management as long as the patient continued therapy.

Forty-five cases with irregular cycles have been treated with 41 cured, two improved, and two failures. Both failures went on to surgery for other reasons. One had an ovarian cyst and the other a degenerated endometrium.

Control series for a group of cases of this kind are difficult. Each case serves as its own control for the period before therapy. Many of the patients relieved of their symptoms ceased medica-

tion and in a few months would have a return of their previous complaints. Thyroid therapy would again correct their menstrual difficulty. In view of the fact that the number of patients who were improved in the present series is almost identical, in proportion, with that of Foster and Thornton in 1939, it seems reasonable to assume that the results are due to the treatment.

Regarding the dosage of thyroid, usually one grain daily is administered for one or two months. If the basal temperature is unusually low, the initial dose may be two grains. If symptoms persist and the basal temperature remains low, the dosage is increased one grain for the following month. The maximum dosage employed has been four grains daily. One case developed symptoms of hyperthyroidism including an elevation of basal temperature, but the symptoms promptly subsided when the dose was reduced.

Improvement in the general health of the patient should be mentioned. Many patients with menstrual disorders complain of fatigue and poor circulation. They require more than the average amount of sleep and yet are tired after a long night's rest. They are nervous, irritable and easily upset by insignificant incidents. Their appetite is often poor, and more of them are underweight than are overweight. Most of these symptoms are relieved by thyroid therapy and although therapy can be stopped after a few months without a return of menstrual symptoms, many patients will return because they do not feel as well as they did during treatment. There would seem no objection to continuation of thyroid therapy as long as the basal temperature is not elevated above normal. It is probably significant that the first case ever treated for thyroid deficiency had to continue medication for 28 years.

SUMMARY

One hundred forty-three cases of functional menstrual disorders have been studied. Dysmenorrhea, excessive bleeding and irregular menstrual cycles are included. The Basal Temperature usually is subnormal. Thyroid therapy relieves the symptoms in about 90 per cent of these cases.

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